

ECON 340: Economics of the Family

TA Session 7

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October 2025

- ▶ Parental Leave Benefits, Household Labor Supply, and Children's Long-run Outcomes
- ▶ When Dad Can Stay Home — AEJ Applied 2024

Ginja, Jans & Karimi (2018). *Parental Leave Benefits, Household Labor Supply, and Children's Long-run Outcomes* (Sweden).

Question: How do *benefit levels* in parental leave (holding duration & job protection fixed) affect

- (i) mothers' and partners' labor supply and household income, and
- (ii) older vs. younger siblings' health & education?

Design: RD at Swedish **Speed Premium (SP)** spacing cutoffs (24 and 30 months) for consecutive births.

Swedish Parental Leave & the Speed Premium

- ▶ 1970s–80s Sweden: paid, job-protected parental leave (PL); wage-replaced benefits (about 90% of prior salary, capped).
- ▶ **Work requirement** (qualifying earnings) usually needed to set the benefit level for the next child.
- ▶ **Speed Premium (SP)**: if births are spaced within a threshold, the mother keeps the *previous* wage-based benefit *without re-qualifying*.
 - ▷ Threshold set to **24 months** (1980–85), then **30 months** (1986–).
 - ▷ Eligibility determined using *expected due date*; revealed during pregnancy.
- ▶ Thus implied that mothers did not have to return to work between births and re-qualify for (higher) PL benefits,

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- ▶ Intuition: SP lowers the opportunity cost of home time *before and after* the subsequent birth.
- ▶ Incentives for mothers to space births in short intervals
- ▶ So mong mothers with at least one child, giving birth to a subsequent child outside of the eligibility spacing interval implies a lower benefit level for leave taken with the new child

- ▶ Linked Swedish registers (parents & children): labor earnings, disposable income, births, health (inpatient), grade 9 GPA, high school eligibility, college by age 24.
- ▶ **Sample:** families with ≥ 2 children; main window ± 24 weeks around the cutoff; second births 1980–89.
- ▶ **Parent outcomes:** mothers'/fathers' labor earnings (pre- and post-birth windows), maternal participation, household disposable income.
- ▶ **Child outcomes:** birth outcomes (younger child), hospitalizations at ages 10/14, education (GPA, high school eligibility, college by 24) for older & younger siblings.

- ▶ **Running variable:** weeks between births 1 and 2 (or 2 and 3 in a higher-parity sample).
- ▶ **Cutoffs:** $c = 104$ weeks (24 months) or $c = 130$ weeks (30 months).
- ▶ **RD (fuzzy ITT):**

$$y_{i,\tau} = \alpha + \mathbf{1}\{t \leq c\}\beta + \mathbf{1}\{t > c\}f_r(t - c) + \mathbf{1}\{t \leq c\}f_l(c - t) + \varepsilon_i,$$

Identification Assumptions

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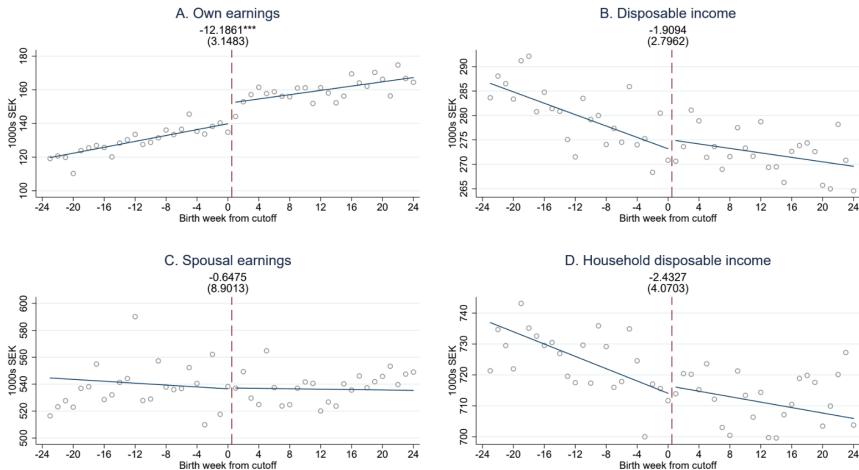
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- ▶ **Local random assignment:** families just below vs. just above the cutoff are comparable in observed and unobserved characteristics.
- ▶ No manipulation of birth timing around the cutoffs.
- ▶ No other policies or events change discontinuously at the cutoffs.

Effect of SP on Earnings and Income

FIGURE 1.

The effect of the SP on spousal labor earnings and household income



Timing of Labor Supply Response

TABLE 3.
The effect of the SP on spousal labor earnings and household income

	(1) Mother's income	(2)	(3) Father's income	(4)	(5) HH disp. income	(6)
Years since birth, 2 nd birth	Pre-birth	Post birth	Pre-birth	Post birth	Pre-birth	Post birth
Treated ($\hat{\beta}$)	-3.838*** (0.729)	-6.756*** (1.232)	0.466 (1.498)	2.920 (3.151)	0.344 (0.964)	6.824*** (1.852)
Observations	71,622	71,290	71,574	71,296	71,950	71,950
Mean of outcome	99.07	129.2	198.3	410.5	249.7	534.3
$\hat{\beta} / \text{Mean}$	-0.0387	-0.0523	0.0024	0.0071	0.0014	0.0128

LS declines pre-birth

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LS declines immediately after- post-birth too - take up of PL

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No effect on fathers' LS

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HH disposable income does not decrease pre-birth - paid maternity leave or government transfers

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Positive effect of SP HH's post-birth disposable income - mothers on PL

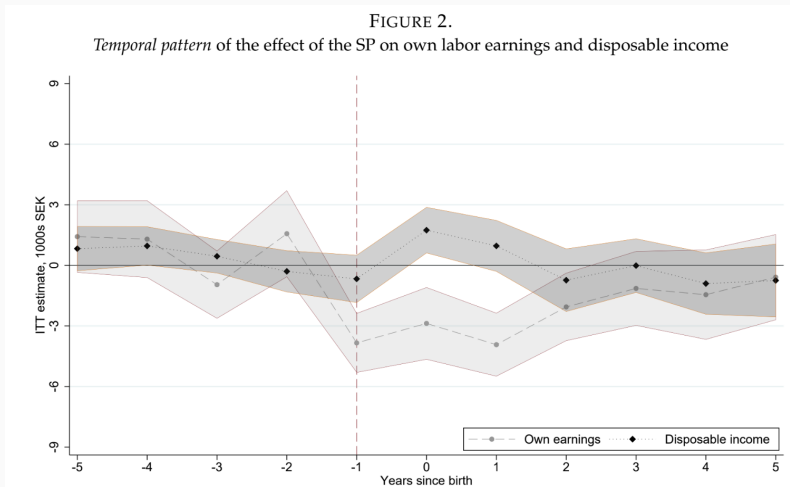
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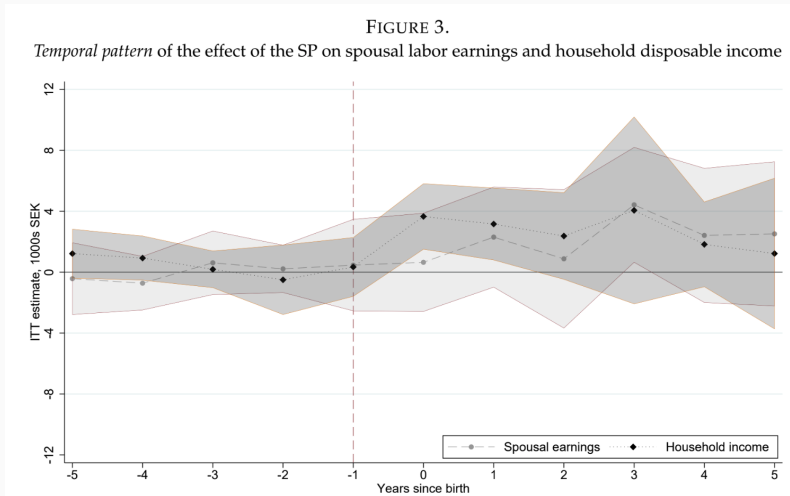
Mothers reduce LS in the year before second birth by nearly 4% relative to baseline earnings

Temporal Effect of SP on Maternal Labor Supply



Pre-treatment placebo

Temporal Effect of SP on Spousal Labor Supply



No effect on fathers' LS - no evidence of substitution

Heterogeneity by Mother's Earnings

TABLE 5.
Heterogeneous effects of the SP on spousal labor earnings and household income by mother's position in the earnings distribution

Years since birth, 2 nd birth	(1)	(2)	(3)	(4)	(5)	(6)
	Mother's income		Father's income		HH disp. income	
	Pre-birth	Post birth	Pre-birth	Post birth	Pre-birth	Post birth
A. Lowest income group (tertile 1)						
Treated ($\hat{\beta}$)	-5.006*** (1.306)	-7.015*** (2.179)	-1.196 (2.476)	-0.125 (6.867)	-2.847** (1.366)	0.415 (3.112)
Observations	20,959	20,810	20,944	20,832	21,074	21,074
Mean of outcome	69.95	87.03	174.5	361.9	225	487.9
$\hat{\beta}$ / Mean	-0.0716	-0.0806	-0.0069	-0.0003	-0.0127	0.0009
B. Middle income group (tertile 2)						
Treated ($\hat{\beta}$)	-3.775*** (1.341) (0.472)	-5.315*** (1.892) (1.194)	-2.318 (2.393) (2.135)	-4.949 (4.453) (2.305)	-2.517* (1.312) (0.909)	1.974 (3.515) (3.029)
Observations	22,402	22,295	22,379	22,302	22,510	22,510
Mean of outcome	89.49	108.3	192.4	395.4	240.3	512.2
$\hat{\beta}$ / Mean	-0.0422	-0.0491	-0.0121	-0.0125	-0.0105	0.0039
C. Highest income group (tertile 3)						
Treated ($\hat{\beta}$)	-3.633* (1.857)	-8.933*** (2.704)	3.155 (3.991)	9.953** (3.928)	4.302*** (1.352)	14.200*** (2.907)
Observations	28,261	28,185	28,251	28,162	28,366	28,366
Mean of outcome	129	178.1	221.3	459.7	276.1	587.6
$\hat{\beta}$ / Mean	-0.0282	-0.0502	0.0143	0.0217	0.0156	0.0242

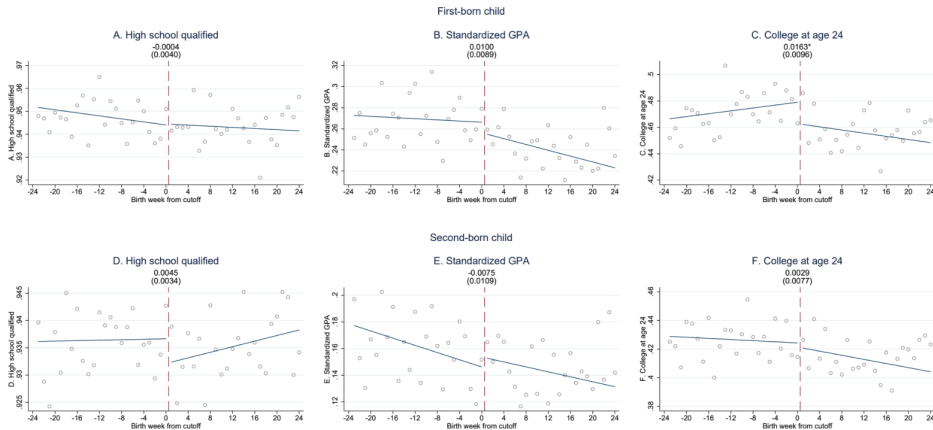
For high-earning couples, SP increases *specialization*: more maternal home time, more paternal market work in the short run.

TABLE 6.
The effect of the SP on the second-born child's outcomes at birth

	(1) Birth weight (grams)	(2) Gestation (weeks)	(3) Low birth weight	(4) Preterm birth	(5) Birth height (cm)	(6) Apgar-1	(7) Apgar-5
Treated ($\hat{\beta}$)	-4.332 (9.518)	0.011 (0.020)	0.001 (0.002)	-0.001 (0.003)	-0.003 (0.048)	-0.007 (0.011)	0.002 (0.011)
Observations	71,685	71,783	71,685	71,783	71,484	71,153	68,318
R-squared	0.000	0.000	0.000	0.000	0.000	0.000	0.001
Mean of outcome	3,598	39.51	0.0206	0.0337	50.61	8.871	9.632
$\hat{\beta} / \text{Mean}$	-0.0012	0.0003	0.0541	-0.0311	0.0000	-0.0008	0.0002

FIGURE 4.
The effect of the SP on children's educational outcomes

Estimated discontinuity at threshold: Children's educational outcomes



Education Outcomes: Older vs Younger Sibling

- ▶ First-born children to mothers with SP eligibility are more likely to attain higher education
- ▶ But that there are no impacts for the younger child
- ▶ Why?

Education Outcomes: Older vs Younger Sibling

- ▶ First-born children to mothers with SP eligibility are more likely to attain higher education
- ▶ But that there are no impacts for the younger child
- ▶ Why? Possibly because the older child is younger at the time of the sibling's birth, and thus receives more exclusive maternal time
- ▶ Time spent with the mother is more important in the early years of life
- ▶ Later child may not benefit as much from the increased maternal time - compete with the older sibling
- ▶ Another penalty : Birth order penalty!

Heterogeneity by Mother's Earnings

TABLE 10.
The effect of the SP on children's education outcomes: heterogeneous effects by mother's position in the earnings distribution

	(1)	(2)	(3)	(4)	(5)	(6)
	Lower birth order child			Higher birth order child		
	High school qualified	Standard. GPA	College at age 24	High school qualified	Standard. GPA	College at age 24
A. Low income group (tertile 1)						
Treated ($\hat{\beta}$)	-0.001 (0.007)	-0.009 (0.017)	0.005 (0.011)	0.006 (0.009)	-0.016 (0.024)	0.000 (0.015)
Observations	16,649	24,998	24,816	20,858	24,843	24,746
Mean of outcome	0.906	-0.0107	0.349	0.891	-0.0895	0.317
$\hat{\beta}$ / Mean	-0.0014	0.879	0.0134	0.0062	0.175	0.000
B. Middle income group (tertile 2)						
Treated ($\hat{\beta}$)	0.000 (0.008)	0.011 (0.017)	0.005 (0.017)	-0.001 (0.008)	-0.016 (0.025)	0.004 (0.012)
Observations	17,926	26,871	26,682	22,580	26,822	26,621
Mean of outcome	0.934	0.132	0.397	0.926	0.0361	0.361
$\hat{\beta}$ / Mean	0.0005	0.0813	0.0132	-0.0007	-0.441	0.0098
C. High income group (tertile 3)						
Treated ($\hat{\beta}$)	0.000 (0.005)	0.045** (0.021)	0.042*** (0.012)	0.004 (0.005)	0.015 (0.022)	0.016** (0.008)
Observations	22,389	32,890	32,600	27,877	32,850	32,465
Mean of outcome	0.968	0.457	0.563	0.963	0.355	0.521
$\hat{\beta}$ / Mean	0.0005	0.0989	0.0745	0.0040	0.0436	0.0314

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Recall that maternal LS responses were similar - quality of maternal care? Complementarities of household income and maternal time?

Conclusions

- ▶ SP has clear incentives for mothers to reduce labor supply before and after birth of the subsequent child
- ▶ No evidence of substitution to fathers' labor supply, except for high-earning couples
- ▶ SP increases children's schooling performance for the older sibling, especially with high-SES mothers

For parents: Higher benefit levels (at fixed duration) shift mothers from market to home in the short run; little long-run earnings impact measured; some short-run *specialization* for high-SES couples.

For children: The *existing* child gains in schooling (GPA/college), especially with high-SES mothers and at the 24-month threshold—consistent with more *exclusive* maternal time replacing lower-quality care.

Distributional angle: The SP can be *regressive* (larger gains among high-SES) and may widen *birth-order gaps*.

- ▶ Parental Leave Benefits, Household Labor Supply, and Children's Long-run Outcomes
- ▶ When Dad Can Stay Home — AEJ Applied 2024

- ▶ Flexibility at the workplace is increasingly valued as an amenity
- ▶ Especially for parents with young children facing unpredictable health and care needs (snow days and sick days!)
- ▶ And even more so for mothers - on call for domestic household needs
- ▶ Goldin (2014) (and others): workplace flexibility could be key to reducing the gender pay gap

What we don't know

- ▶ The impact of workplace flexibility on health costs of mothers
- ▶ Fathers' demand for workplace flexibility and their spillover effects on maternal wellbeing
- ▶ Role of moral hazard in paid family leave policies and the role of maternal health benefits

Persson & Rossin-Slater (2024). *When Dad Can Stay Home: Fathers' Workplace Flexibility and Maternal Health*, *AEJ: Applied Economics*, 16(4):186–219.

- ▶ **Question:** Does granting *fathers* flexible access to leave improve **maternal postpartum health**?
- ▶ **Policy:** Sweden's “**Double Days**” (Jan 1, 2012) — up to **30 additional days** of *simultaneous* full-time parental leave in the child's first year (beyond the baseline 10 days).
- ▶ **Design:** RD–DiD using *share of days eligible* in the first month post-birth around the cutoff
- ▶ **Preview:** More paternal flexibility $\Rightarrow$ **fewer maternal complications** and **lower anti-anxiety medicine use**

Institutional Background

- ▶ 1974: Sweden introduced gender-neutral paid parental leave
- ▶ Pre-reform: parents could not take paid leave *together* –except **10 baseline days** in the first 60 days post birth. Why?
- ▶ **2012 reform:** up to **30 extra** *intermittent* simultaneous leave days in the first year; *no change* to total leave allotment
- ▶ No advance employer notice required for the second leavetaker if the mother is sick;
- ▶ Effectively increases **temporal flexibility for fathers** on how to allocate the timing of their leave

Maternal Health Outcomes (Pre-Reform)

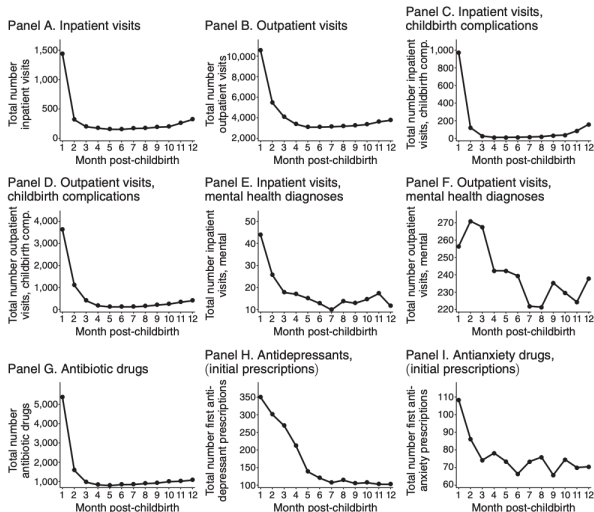


FIGURE 1. FREQUENCY OF MATERNAL HEALTH ISSUES BY MONTH POST-CHILDBIRTH, 2008–2011 BIRTHS

- ▶ Birth data: all Swedish births 2000-2016 and characteristics, identify singleton births
- ▶ Parental leave claims - all leave spells
- ▶ Maternal health outcomes - inpatient and outpatient visits, prescriptions filled

Design Sketch: RD-DD Around Jan 1, 2012

- ▶ The goal: isolate variation in **father's leave flexibility** around Jan 1, 2012 reform to study causal effect on maternal health
- ▶ Key idea: births just before vs just after Jan 1, 2012 differ in **eligibility for simultaneous leave** in the first month postpartum
- ▶ Eg: Dec 31, 2011 birth: only 29 days eligible in first month – share $29/30 = 0.97$
- ▶ Jan 1, 2012 birth: full 30 days eligible – share $30/30 = 1.00$
- ▶ Compare outcomes of parents of children born in January - March 2012 and October - December 2011 to those born in same months in 2010, 2009, 2008

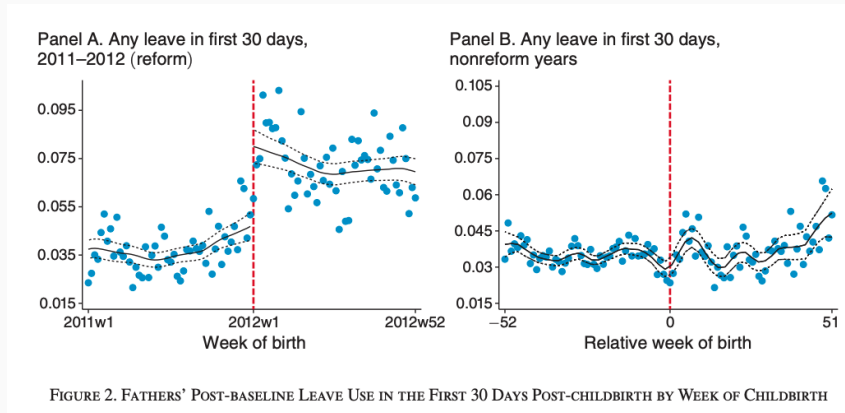
► Estimate:

$$Y_{idp} = \alpha_0 + \alpha_1 \text{ShareDaysEligible}_{idp} + \alpha_2 1\{d \geq c\} + f(d-c) + 1\{d \geq c\} \times f(d-c) + \theta_p + \varepsilon_{idp}$$

Design Sketch: RD-DD Around Jan 1, 2012

- ▶ What's different about this design from the usual RDD?
 - ▷ Running variable is continuous (share of days eligible in month 1)
 - ▷ Treatment is not a binary jump at the cutoff, but a change in the slope of the running variable at the cutoff
- ▶ Why use the Difference-in-Differences?
 - ▷ Controls for seasonality in births and health outcomes
- ▶ What's the identifying assumption?
 - ▷ No other changes at the date cutoff that differentially affect maternal health outcomes

First Stage: Fathers Use More Leave in Month 1

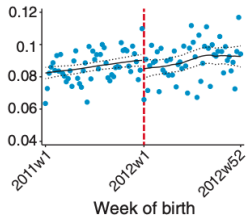


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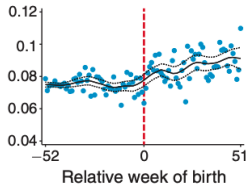
- ▶ Full eligibility $\Rightarrow$ father uses **any post-baseline leave** + **3.9 p.p.** (+92% at mean).
- ▶ Seasonal variation in leave: needed the non-reform years and the DD design
- ▶ Leave starts to increase at the end of 2011 - consistent with them being eligible

Maternal Health: Childbirth Complications

Panel A. Childbirth complications, 2011–2012 (reform)

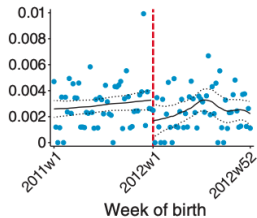


Panel D. Childbirth complications, nonreform years

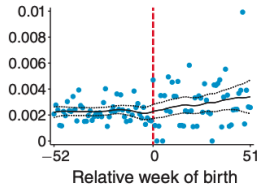


Maternal Health: Anxiety Medications

Panel B. Antianxiety drug,
2011–2012 (reform)

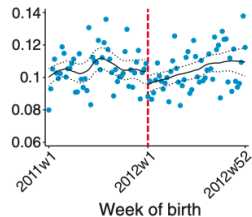


Panel E. Antianxiety drug,
nonreform years

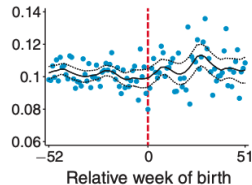


Maternal Health: Antibiotic Drugs

Panel C. Antibiotic drug,
2011–2012 (reform)



Panel F. Antibiotic drug,
nonreform years



In Regression Form

TABLE 3—EFFECTS OF “DOUBLE DAYS” REFORM ON MATERNAL HEALTH OUTCOMES IN FIRST 30 DAYS POST-CHILDBIRTH IN INPATIENT AND OUTPATIENT DATA

	Any	Diagnosis Categories		
		Childbirth comp.	Mental	External/Counseling
<i>A. All first births</i>				
Share days eligible in days 1–30 post-birth	–0.0103 [0.00677]	–0.00976 [0.00484]	0.000336 [0.00125]	–0.00135 [0.000586]
Romano-Wolf <i>p</i>	{0.267}	{0.178}	{0.792}	{0.079}
Dep. var mean	0.185	0.0799	0.00492	0.00125
<i>N</i>	82,558	82,558	82,558	82,558
<i>RD-DD drop December births</i>				
Reform × birth Jan–Mar	–0.0128 [0.00706]	–0.0130 [0.00504]	0.00109 [0.00128]	–0.00120 [0.000592]
Romano-Wolf <i>p</i>	{0.198}	{0.050}	{0.455}	{0.139}
Dep. var mean	0.185	0.0801	0.00496	0.00119
<i>N</i>	69,953	69,953	69,953	69,953
<i>B. Mothers with medical history</i>				
Share days eligible in days 1–30 post-birth	–0.00400 [0.0132]	–0.0197 [0.00955]	0.000567 [0.00388]	–0.00194 [0.00114]
Romano-Wolf <i>p</i>	{0.941}	{0.238}	{0.941}	{0.347}
Dep. var mean	0.229	0.0972	0.0143	0.00159
<i>N</i>	23,935	23,935	23,935	23,935
<i>RD-DD drop December births</i>				
Reform × birth Jan–Mar	–0.00567 [0.0139]	–0.0208 [0.00999]	0.00266 [0.00400]	–0.00133 [0.00107]
Romano-Wolf <i>p</i>	{0.772}	{0.168}	{0.772}	{0.505}
Dep. var mean	0.228	0.0983	0.0146	0.00153
<i>N</i>	20,230	20,230	20,230	20,230

- ▶ Maternal **inpatient/outpatient** visit for childbirth complications: **1.0 p.p. (12%)**.
- ▶ Mothers with pre-birth medical histories: **2p.p. (20%)**.

TABLE 4—EFFECTS OF “DOUBLE DAYS” REFORM ON MATERNAL HEALTH OUTCOMES IN FIRST 30 DAYS POST-CHILDBIRTH IN PRESCRIPTION DRUG DATA

	Any prescription drug	Any anxiety drug	Any antidepressant drug	Any painkiller	Any antibiotic
<i>A. All first births</i>					
Share Days Eligible in days 1-30 Post-Birth	−0.00867 [0.00725]	−0.00172 [0.000899]	−0.000999 [0.00160]	−0.00385 [0.00398]	−0.0145 [0.00519]
Romano-Wolf <i>p</i>	{0.475}	{0.188}	{0.515}	{0.515}	{0.030}
Dep. var mean	0.233	0.00240	0.00855	0.0579	0.101
<i>N</i>	82,558	82,558	82,558	82,558	82,558
<i>RD-DD Drop December Births</i>					
Reform × Birth Jan–Mar	−0.0124 [0.00758]	−0.00147 [0.000931]	−0.000783 [0.00168]	−0.00346 [0.00416]	−0.0186 [0.00543]
Romano-Wolf <i>p</i>	{0.376}	{0.376}	{0.723}	{0.703}	{0.030}
Dep. var mean	0.233	0.00247	0.00875	0.0583	0.101
<i>N</i>	69,953	69,953	69,953	69,953	69,953
<i>B. Mothers with medical history</i>					
Share Days Eligible in days 1-30 Post-Birth	−0.00676 [0.0141]	−0.00487 [0.00255]	−0.00526 [0.00515]	−0.00341 [0.00831]	−0.0178 [0.0103]
Romano-Wolf <i>p</i>	{0.851}	{0.257}	{0.584}	{0.851}	{0.257}
Dep. var mean	0.290	0.00618	0.0280	0.0804	0.125
<i>N</i>	23,935	23,935	23,935	23,935	23,935

RD-DD Drop December Births

In Regression Form

TABLE 4—EFFECTS OF “DOUBLE DAYS” REFORM ON MATERNAL HEALTH OUTCOMES IN FIRST 30 DAYS POST-CHILDBIRTH IN PRESCRIPTION DRUG DATA

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<i>N</i>	69,953	69,953	69,953	69,953	69,953
<i>B. Mothers with medical history</i>					
Share Days Eligible in days 1-30 Post-Birth	−0.00676 [0.0141]	−0.00487 [0.00255]	−0.00526 [0.00515]	−0.00341 [0.00831]	−0.0178 [0.0103]
Romano-Wolf <i>p</i>	{0.851}	{0.257}	{0.584}	{0.851}	{0.257}
Dep. var mean	0.290	0.00618	0.0280	0.0804	0.125
<i>N</i>	23,935	23,935	23,935	23,935	23,935
<i>RD-DD Drop December Births</i>					
Reform × Birth Jan–Mar	−0.0120 [0.0148]	−0.00401 [0.00263]	−0.00471 [0.00547]	−0.00183 [0.00874]	−0.0235 [0.0109]
Romano-Wolf <i>p</i>	{0.822}	{0.455}	{0.822}	{0.822}	{0.158}
Dep. var mean	0.293	0.00623	0.0287	0.0812	0.126
<i>N</i>	20,230	20,230	20,230	20,230	20,230

In Regression Form

TABLE 4—EFFECTS OF “DOUBLE DAYS” REFORM ON MATERNAL HEALTH OUTCOMES IN FIRST 30 DAYS POST-CHILDBIRTH IN PRESCRIPTION DRUG DATA

	Any prescription drug	Any anxiety drug	Any antidepressant drug	Any painkiller	Any antibiotic
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<i>N</i>	82,558	82,558	82,558	82,558	82,558
<i>RD-DD Drop December Births</i>					
Reform × Birth Jan–Mar	−0.0124 [0.00758]	−0.00147 [0.000931]	−0.000783 [0.00168]	−0.00346 [0.00416]	−0.0186 [0.00543]
Romano-Wolf <i>p</i>	{0.376}	{0.376}	{0.723}	{0.703}	{0.030}
Dep. var mean	0.233	0.00247	0.00875	0.0583	0.101
<i>N</i>	69,953	69,953	69,953	69,953	69,953
<i>B. Mothers with medical history</i>					
Share Days Eligible in days 1-30 Post-Birth	−0.00676 [0.0141]	−0.00487 [0.00255]	−0.00526 [0.00515]	−0.00341 [0.00831]	−0.0178 [0.0103]
Romano-Wolf <i>p</i>	{0.851}	{0.257}	{0.584}	{0.851}	{0.257}
Dep. var mean	0.290	0.00618	0.0280	0.0804	0.125
<i>N</i>	23,935	23,935	23,935	23,935	23,935
<i>RD-DD Drop December Births</i>					
Reform × Birth Jan–Mar	−0.0120 [0.0148]	−0.00401 [0.00263]	−0.00471 [0.00547]	−0.00183 [0.00874]	−0.0235 [0.0109]
Romano-Wolf <i>p</i>	{0.822}	{0.455}	{0.822}	{0.822}	{0.158}
Dep. var mean	0.293	0.00623	0.0287	0.0812	0.126
<i>N</i>	20,230	20,230	20,230	20,230	20,230

- ▶ Maternal anti-anxiety prescription: **0.2 p.p. (72%)**.
- ▶ Maternal antibiotic prescription: **1.5 p.p. (14%)**.

Mechanism: Targeted Co-Presence on Medical Days

TABLE 5: EFFECT OF “DOUBLE DAYS” REFORM ON THE LIKELIHOOD OF FATHER TAKING LEAVE ON DAYS WHEN MOTHER NEEDS MEDICAL CARE

	Father takes leave when mother gets medical care	
	Share days eligible model	“Doughnut” RD-DD
<i>A. All first births</i>		
Share days eligible in first 30 days	0.00495 [0.00200]	
Reform $\times$ birth Jan–Mar		0.00577 [0.00209]
Dep. var mean	0.0125	0.0130
<i>N</i>	82,558	69,953
<i>B. Mothers with medical history</i>		
Share days eligible in first 30 days	0.0128 [0.00451]	
Reform $\times$ birth Jan–Mar		0.0134 [0.00479]
Dep. var mean	0.0198	0.0209
<i>N</i>	23,935	20,230

Mechanism: Targeted Co-Presence on Medical Days

- ▶ Eligibility increases the probability the **father takes leave on a day** the mother has a medical encounter by **+0.5 p.p. (+40%)**
- ▶ **+1.3 p.p. (+65%)** if the mother has a pre-birth medical history.
- ▶ Fathers' flexibility appears to facilitate **prompt care** or support on days with domestic health shocks.

Conclusion

- ▶ Mother's environment at home can have significant influence on her well-being
- ▶ Mothers bear the majority of the cost of a lack of workplace flexibility:
 - ▷ Both directly - career costs of family formation
 - ▷ But also indirectly - fathers' inflexible leave exacerbates maternal health problems
- ▶ Effects on these maternal health outcomes are larger in both absolute and relative terms for mothers with a pre-birth medical history
- ▶ Allow families to decide when it's optimal for both parents to be home - even if that means simultaneous leave
- ▶ Aside: the United States is the only high-income country with a national PFL policy

Interpretation

- ▶ Households face **stochastic domestic shocks** (e.g., maternal infections, fatigue, stress).
- ▶ Flexibility expands the feasible set: **occasion-specific** paternal leave generates real health gains even with *modest average days used*.
- ▶ **Collective model:** Expanding one spouse's choice set can raise the other's welfare via intra-household reoptimization.
- ▶ Complements child-penalty literature by focusing on **health spillovers**, not just labor supply.
- ▶ Baseline rule restricting simultaneous leave to 10 days likely **forgoes health benefits** to deter moral hazard; need to quantify the trade-off
- ▶ Policy design matters: Timing, simultaneity, and day-level granularity can be as important as total leave weeks.